

## Title

TERM-HYP: Determining Optimal Delivery Timing in Hypertensive Pregnancy--A Multi-Center Randomized Protocol

## Abstract

Hypertensive disorders of pregnancy complicate approximately 7% of gestations. Women who maintain stable blood pressure control and reach term face a clinical decision regarding delivery timing: immediate induction or expectant management pending spontaneous labor. Early planned delivery may mitigate maternal hypertensive crises and adverse events but carries potential for increased neonatal complications. This protocol describes a 50-center pragmatic randomized trial comparing two delivery strategies in women with well-controlled hypertension at term. Eligible participants are those randomized at 36-37 weeks gestation with singleton pregnancies. The intervention arm undergoes planned induction at 38 weeks; the comparator arm receives standard obstetric care. Co-primary outcomes address both maternal and neonatal safety; secondary outcomes include cesarean rates and cost-effectiveness. Follow-up extends to 6 weeks postpartum.